CASE STUDY 02

National Health Insurance Risk Review: From Ghost Patients to a Fake Vendor Network

Risk review of claims and procurement controls for a national public health insurance authority.

Archish Chugh

Internal Audit · Internal Controls · GRC · Fraud Investigations · 2026

chugharchish@gmail.com · linkedin.com/in/archish-chugh

Client names and identifying details have been withheld. Prepared by Archish Chugh for portfolio purposes; this is not an EY publication and does not represent the views of any client.

CASE STUDY 02 · HEALTHCARE · RISK-BASED REVIEW

National Health Insurance Risk Review: From Ghost Patients to a Fake Vendor Network

Risk review for a national public health insurance authority, covering claims and procurement controls. Traced ghost patients and false billing, and followed the money to a fake vendor network that was later shut down.

Archish Chugh · chugharchish@gmail.com · linkedin.com/in/archish-chugh

CLIENT TYPE	ROLE	APPROACH	SCOPE
National public health insurance authority	Senior Analyst, EY	Risk-based review	Claims & procurement

EXECUTIVE SUMMARY

SITUATION

A national public health insurance authority wanted a review of its claims and procurement controls.

CHALLENGE

Fraud was spread across enrolment, referrals, billing and hospital procurement.

APPROACH

Ranked risks, ran analytics on the full claims data, checked patient records and followed payments through intercompany deposits and counterparty records.

RESULT

Hundreds of suspect claims flagged. Kickbacks, shell companies and a fake vendor network found; the network was later shut down.

4	6	Hundreds	Shut down
fraud schemes found, from enrolment through to procurement	stages of the money trail tested, from enrolment to vendor payment	of suspect claims and ghost-patient registrations flagged	the fake vendor network found in the review

01

Background

National health insurance schemes pay empanelled hospitals for treating registered beneficiaries. Each step, from enrolment to admission, pre-authorisation, claim, payout and the hospital's own purchasing, is a point where money can be taken. The authority wanted to know where its controls were failing and who was benefiting.

02

Approach



1 **Planning**
Mapped the claims cycle and the providers' procurement cycle. Ranked the risks by likelihood and impact, and put fieldwork time into the highest-risk providers and processes rather than spreading it evenly.

2 **Data analytics**
Tested the full claims population for red flags: identity mismatches, impossible treatment patterns, clusters of claims from one referral source, and directors who also appeared on vendor records.

3 **Fieldwork**
Checked patient records against registrations, followed payments through intercompany deposits and counterparty records, and verified whether vendors really existed and operated.

4 **Root cause**
For each finding, identified the control that failed (verification, segregation, disclosure or vendor due diligence) and explained how it was bypassed.

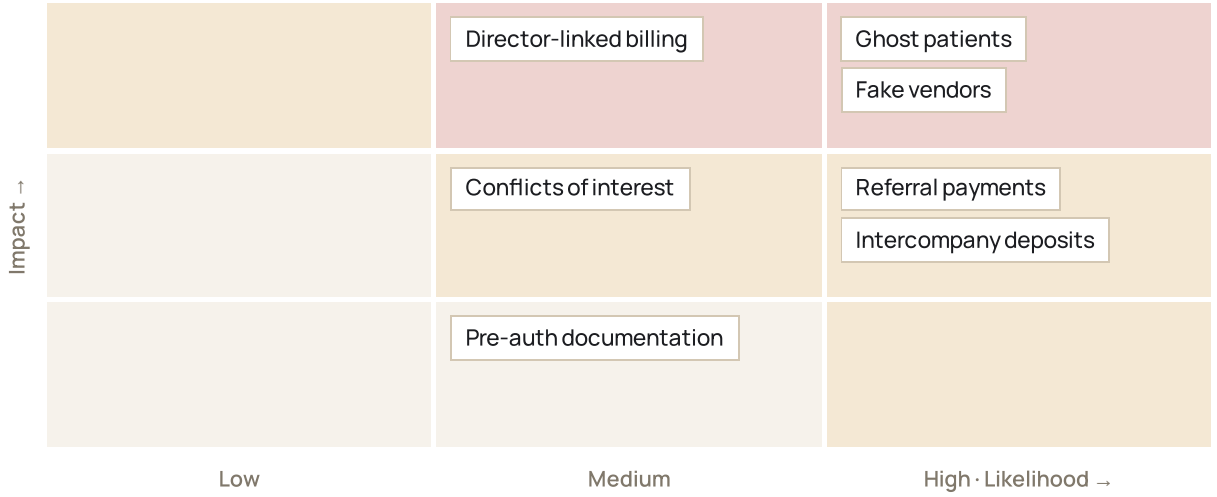
5 **Reporting**
Rated the findings, supported each with evidence, and gave practical recommendations. Worked with the authority on what to do first.

03

Risk Assessment

Exhibit 1: Planning-Stage Risk Assessment

Qualitative ratings

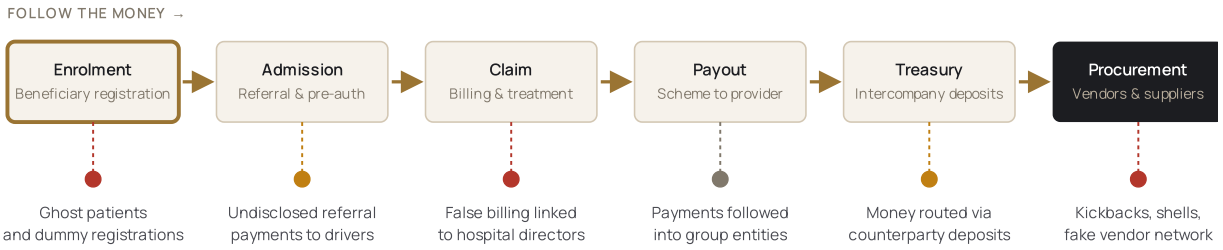


04

Following the Money

Exhibit 2: Findings at Each Stage of the Money Trail

Click a stage for detail



CRITICAL HIGH TRACED, SUPPORTS OTHER FINDINGS

05

Findings

SCHEME	HOW IT WORKED	CONTROL THAT FAILED	RAG RATING
Ghost patients & dummy registrations	Beneficiaries who did not exist, or were not treated, were registered and claimed for.	Identity verification at enrolment and admission	CRITICAL

SCHEME	HOW IT WORKED	CONTROL THAT FAILED	RAG RATING
Director-linked false billing	False claims traced to entities connected to the hospital's directors.	Conflict-of-interest declarations; claim audit	CRITICAL
Undisclosed referral payments	Payments were made to ambulance drivers to bring patients to particular hospitals, and were never disclosed.	Referral monitoring; disclosure requirements	HIGH
Procurement kickbacks & fake vendors	Money moved through intercompany deposits to shell companies and a fake vendor network.	Vendor due diligence; segregation of duties in procure-to-pay	CRITICAL

RAG key: CRITICAL (RED) HIGH (AMBER) MODERATE (GREEN)

06

Outcome and Recommendations

> The fake vendor network found in the review was later shut down. The authority received evidence to act against the providers involved, and recommendations to stop the same schemes across its network.

Recommendations

RECOMMENDATION	ADDRESSES	PRIORITY
Verify beneficiary identity at enrolment and again at admission	Ghost patients and dummy registrations	HIGH
Require disclosure of referral arrangements and director conflicts of interest	Referral payments; director-linked billing	HIGH
Introduce vendor due diligence and procure-to-pay segregation of duties for empanelled providers	Kickbacks and fake vendors	HIGH
Run analytics-based red flags for ongoing claims monitoring	All schemes	MEDIUM

KEY TAKEAWAYS

1. Fraud at one stage is often linked to fraud at another, so payments need to be followed across the whole cycle.
2. Referral payments and hospital procurement should be in scope, not only claims.
3. Running analytics on the full claims data shows where to focus fieldwork.

UAE

Relevance to UAE Roles

Mandatory health insurance in Dubai and Abu Dhabi faces the same risks: provider fraud, referral incentives and upcoding. DHA and DoH Abu Dhabi expect insurers, TPAs and providers to have fraud controls in place. This work is relevant to insurer internal audit, payer fraud and healthcare compliance roles.